

Soren Acquisition Corp. — U.S. Healthcare SPAC Target Screening Report

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Purpose: first-pass screening of U.S. private healthcare companies as potential SPAC business-combination targets

Data policy: public/free sources only

Output type: agent-generated SPAC target screening report, not investment advice

Scope limitation. This report is based on public-source screening. Private-company revenue, EBITDA, leverage, shareholder intentions, rollover capacity, litigation exposure, and audit readiness are not fully visible from current public materials. Confirmed financial metrics are shown only where current public disclosure supports them; otherwise the gap is carried into follow-on diligence.

1. Executive Thesis

Soren's most credible first-wave opportunity set remains **sponsor-backed provider-services and specialty behavioral-health platforms** where operating scale, reimbursement exposure, consolidation logic, and public-market messaging can be diligenced with discipline rather than guessed at.

Within that frame, **physician-services / MSO platforms still screen best**, particularly when they combine visible operating footprint, sponsor ownership, and a believable use for public equity currency. Behavioral-health platforms remain highly relevant, but they demand a tighter underwriting standard around billing quality, staffing durability, outcomes support, and disclosure readiness.

The current screen identifies **3 names for active validation**:

1. **US Heart and Vascular**
2. **Centria Healthcare / Centria Autism**
3. **Hopebridge**

The remaining shortlisted names are still strategically relevant, but they should not be treated as equivalent outreach priorities until route timing, scale visibility, and public-company readiness become more concrete.

This ranking should be read as **current validation priority**, not a final transaction recommendation. Public evidence supports the broad strategic direction, but current public sources

do not establish seller intent, PIPE receptivity, management rollover, or audit readiness with the level of confidence required for sponsor engagement.

2. Buyer Conviction Map

Soren capability / profile area	Acquisition relevance	Screening implication
Healthcare operating / provider-services orientation	Helps evaluate multi-site operations, physician alignment, staffing, and integration risk.	Favors real operating platforms over narrative-only healthtech assets.
Reimbursement and policy familiarity	Useful where payer mix, billing discipline, and regulatory detail affect valuation.	Prioritize businesses whose reimbursement complexity can actually be underwritten.
Public-company and capital-markets relevance	A SPAC combination requires a credible investor story, governance discipline, and route feasibility.	Favor targets with a public-market narrative tied to scale, consolidation, and capital use.
Healthcare network / board credibility	Can improve diligence access and judgment in regulated care-delivery settings.	Focus on targets where Soren adds underwriting credibility, not just capital.
SPAC structure as a tool, not a default answer	SPAC can provide public currency and liquidity, but only when the route is genuinely competitive.	Do not confuse an attractive company with an automatically attractive SPAC candidate.

The practical implication is straightforward: Soren should spend its earliest validation effort on healthcare-services businesses where sector knowledge materially improves underwriting quality and where the public-market route can be defended on fundamentals rather than on theme alone.

3. Subsector Priority Map

Priority	Subsector	Why it matters	Preferred target traits	Main reasons to downgrade
1	Physician services / MSO with specialty or payer interface	Best balance of operating tangibility, comparable-company logic, consolidation runway, and Soren relevance.	Specialty platforms with visible sites/providers, sponsor ownership, and a real use for public currency.	Subscale roll-ups, weak physician retention, poor acquisition accounting, no differentiated public story.
2	Behavioral health with cleaner reimbursement footing	Strong demand and consolidation logic, but higher diligence burden around billing, staffing, and outcomes.	Autism ABA and specialty behavioral-health models with payer coverage and quality infrastructure.	Billing controversy, out-of-network exposure, clinician shortages, poor outcomes support.
3	Value-based care / post-acute / senior-care adjacencies	Strategically close to Soren when economics are proven.	Platforms with measurable risk-contract economics, care-management ROI, and manageable valuation.	Too large, too venture-valued, or too dependent on opaque risk-bearing accounting.
4	Healthcare staffing / workforce services	Potentially relevant but less differentiated unless the platform has specialized clinical value.	Specialized staffing or workforce infrastructure with a durable margin profile.	Pure labor arbitrage and weak public-market narrative.
5	AI-enabled clinical / data platforms	Useful as an embedded capability, but weaker as a standalone first-wave acquisition thesis.	Tools with demonstrated ROI, compliance discipline, and real revenue.	Narrative-led AI without proven economics or clinical validation.

The ranking above is less about generic healthcare attractiveness and more about where Soren can form a differentiated acquisition view with enough observable evidence to matter.

4. Active Opportunity Set

The most relevant opportunity set remains **sponsor-backed or institutionally financed healthcare-services platforms** that are mature enough to support a public-company narrative but may not have an obvious conventional IPO path. The best current candidates tend to share several features: visible operating scale, sponsor or institutional ownership, a business model public investors can understand, and a plausible use for public currency or incremental balance-sheet capital.

Route timing still matters. Sponsor ownership on its own is not enough. A stronger current screen gives more weight to signs such as strategic repositioning, capital structure pressure, financing needs, acquisition-currency logic, or other evidence that the timing question may be live rather than theoretical.

4.1 Universe Funnel

The table below summarizes how the current run narrows from the named universe into the active validation set.

Stage	Count	Screening logic
Initial named public-source universe	19	Named companies captured in the current public-source universe table for this run.
Names retained for shortlist modeling	5	Names advanced from the current universe into shortlist modeling after initial relevance and fit screening.
Candidates carried into detailed healthcare-services review	5	Names carried into deeper ownership, timing, and transaction-feasibility review.
Qualified priority shortlist	3	Names that currently clear the screen for active validation rather than provisional monitoring.
Current active-validation names	3	Names prioritized for near-term SPAC validation based on the current evidence set.

The funnel should be read as the factual shape of the current screen, not as a substitute for company-level underwriting.

5. Target Shortlist

Only the qualified names currently warrant **active validation as near-term SPAC candidates**. The remaining names are still useful coverage names, but they sit in a provisional bucket until route quality, scale support, and readiness evidence improve.

5.1 Evidence-tag legend

The memo uses lightweight evidence tags on key judgment sentences so that stronger and weaker forms of support are not presented as though they were equally hard.

Tag	Meaning
[FACT-A]	Original or regulatory public source; highest-confidence public fact.
[FACT-B]	Credible public fact from company, sponsor, or reputable coverage.
[INF-S]	Strong inference supported by multiple public signals.
[INF-W]	Weaker inference or longer logic chain; not enough to clear a critical gate by itself.
[UNK]	Public information missing.
[DD]	Requires confirmatory diligence even if some public signal exists.

5.2 Comparison snapshot

Candidate	Location / subsector	Revenue est.	EBITDA est.	EV est.	Current status	Current conclusion
US Heart and Vascular	Nashville, Tennessee — Cardiology MSO / specialty physician services / value-based specialty care	Not publicly disclosed	Not publicly disclosed	Not publicly disclosed	Qualified priority shortlist	Highest-priority validation candidate

Candidate	Location / subsector	Revenue est.	EBITDA est.	EV est.	Current status	Current conclusion
Centria Healthcare / Centria Autism	Farmington Hills, Michigan — Autism ABA / behavioral health services	Not publicly disclosed	Not publicly disclosed	Not publicly disclosed	Qualified priority shortlist	Priority candidate, conditional on reimbursement and compliance hardening
Hopebridge	Indianapolis, Indiana — Autism ABA / pediatric behavioral health	Not publicly disclosed	Not publicly disclosed	Not publicly disclosed	Qualified priority shortlist	Priority research candidate, but more scale-sensitive than the top two
Eating Recovery Center / Pathlight Mood & Anxiety Center	Denver, Colorado — Eating disorder and mood/anxiety treatment	Not publicly disclosed	Not publicly disclosed	Not publicly disclosed	Provisional carry-forward	Keep in review, but not yet an actionable shortlist name
Anne Arundel Dermatology	Linthicum, Maryland — Dermatology MSO / specialty physician services	Not publicly disclosed	Not publicly disclosed	Not publicly disclosed	Provisional carry-forward	Useful comparison and backup name, but not yet in the actionable top tier

5.3 US Heart and Vascular

Current status: Qualified priority shortlist

Current conclusion: Highest-priority validation candidate

Field	Current read
Headquarters & subsector	Nashville, Tennessee — Cardiology MSO / specialty physician services / value-based specialty care

Field	Current read
Financial view	No confirmed public revenue, EBITDA, or EV. Best public operating-scale proxy: Public materials and sector coverage reference 200+ sites and 500+ cardiologists plus a specialty platform spanning practice management and value-based-care initiatives.
Ownership context	Ares Management Private Equity Group and Oak HC/FT-backed; Rubicon Founders partnership publicly announced
Current route read	SPAC is competitive, but sponsor recap / sponsor-to-sponsor sale remain credible alternatives
Evidence profile	FACT coverage 45%

Bottom-line read. This is the cleanest current blend of visible operating density, sponsor-backed ownership, and a believable specialty-care equity story, which is why it sits at the top of the screen. [INF-S] The remaining gap is not strategic relevance but hard financial and readiness confirmation. [DD]

Basic company characteristics. US Heart and Vascular is based in Nashville, Tennessee and operates in cardiology MSO / specialty physician services / value-based specialty care. [FACT-B] Public materials and sector coverage reference 200+ sites and 500+ cardiologists plus a specialty platform spanning practice management and value-based-care initiatives. [FACT-B] The operating picture is concrete enough to treat the company as a real platform rather than a thematic placeholder. [INF-S]

Financial indicators. Current public sources do not disclose confirmed revenue, EBITDA, or EV. [UNK] The report therefore does not infer valuation or earnings ranges from screening-level signals. [DD] In provider services, visible footprint and clinician density matter because they make it easier to judge whether the platform has real operating depth rather than just roll-up narrative. [INF-S] What still cannot be answered from public materials is whether margin quality, leverage, and transaction perimeter would leave a clean SPAC-sized deal. [DD] Next-round diligence should confirm recent revenue EBITDA debt acquisition history and cardiology MSO comp multiples. [DD]

Control / ownership structure. Current public materials indicate Ares Management Private Equity Group and Oak HC/FT-backed; Rubicon Founders partnership publicly announced. [FACT-B] Visible sponsor ownership lowers the odds that this is an orphaned asset and makes a structured process more plausible, but it still does not prove current exit intent. [INF-S] Current public sources do not show a banker-led process, formal sale process, or other direct evidence that transaction timing is already live. [UNK][DD]

Management / maturity / readiness. Finance-team visibility is unknown. [UNK] Audit risk currently reads medium, and the next diligence question is CFO/controller, audit history, acquisition accounting, VBC contract accounting. [DD] The gating issue here is not presentation polish; it is whether site-level economics, physician compensation, bolt-on accounting, and disclosure controls would stand up in a public filing. [DD]

Strategic synergy. Ranks first because it pairs visible specialty-care scale with sponsor ownership and a sharper public-market story than the other MSO names; if the financials hold, this is the easiest current asset to imagine in a disciplined SPAC narrative. [FACT-B][INF-S] Soren's edge is easiest to defend in tangible care-delivery models where operating density, specialty positioning, and consolidation logic can be underwritten directly. [INF-S] The practical question is whether Soren would be adding underwriting advantage to this asset, not just capital. [INF-S]

Market & transaction feasibility. Current timing evidence is 2021 platform formation; 2024 Rubicon VBC partnership. [FACT-B][INF-S] The public-currency case reads as plausible. [INF-S] The best current route read is that SPAC is competitive, but sponsor recap / sponsor-to-sponsor sale remain credible alternatives. [INF-S] Today that supports active validation, but it still falls short of true sponsor-engagement readiness. [DD]

Principal risks. Financial scale, physician retention, payor concentration, VBC economics, leverage, acquisition accounting and public-company readiness

Priority diligence focus. Confirm current revenue / EBITDA, leverage, physician alignment, value-based-care economics, owner intentions, and public-company readiness.

5.4 Centria Healthcare / Centria Autism

Current status: Qualified priority shortlist

Current conclusion: Priority candidate, conditional on reimbursement and compliance hardening

Field	Current read
Headquarters & subsector	Farmington Hills, Michigan — Autism ABA / behavioral health services
Financial view	No confirmed public revenue, EBITDA, or EV. Best public operating-scale proxy: Public website shows a multi-state autism-services platform spanning diagnostic services plus in-home and center-based ABA care, with additional pediatric nursing / catastrophic-injury service lines also visible.
Ownership context	Thomas H. Lee Partners-backed; prior Lorient/Martis Capital ownership publicly referenced

Field	Current read
Current route read	SPAC is plausible, but strategic sale and sponsor-to-sponsor sale may still be more natural default paths
Evidence profile	FACT coverage 35%

Bottom-line read. The company matters because it combines real care-delivery scale with sponsor ownership in a demand-heavy autism-services category. [FACT-B][INF-S] It ranks below the top provider-services name because reimbursement, staffing, and compliance diligence are more likely to decide the outcome than the headline growth story. [DD]

Basic company characteristics. Centria Healthcare / Centria Autism is based in Farmington Hills, Michigan and operates in autism ABA / behavioral health services. [FACT-B] Public website shows a multi-state autism-services platform spanning diagnostic services plus in-home and center-based ABA care, with additional pediatric nursing / catastrophic-injury service lines also visible. [FACT-B] The operating picture is concrete enough to treat the company as a real platform rather than a thematic placeholder. [INF-S]

Financial indicators. Current public sources do not disclose confirmed revenue, EBITDA, or EV. [UNK] The report therefore does not infer valuation or earnings ranges from screening-level signals. [DD] In behavioral health, visible footprint matters because reimbursement quality and staffing durability cannot be underwritten if the operating base itself is still fuzzy. [INF-S] What still cannot be answered from public materials is whether margin quality, leverage, and transaction perimeter would leave a clean SPAC-sized deal. [DD] Next-round diligence should confirm latest revenue EBITDA debt payer mix and ABA transaction comps. [DD]

Control / ownership structure. Current public materials indicate Thomas H. Lee Partners-backed; prior Lorient/Martis Capital ownership publicly referenced. [FACT-B] Visible sponsor ownership lowers the odds that this is an orphaned asset and makes a structured process more plausible, but it still does not prove current exit intent. [INF-S] Current public sources do not show a banker-led process, formal sale process, or other direct evidence that transaction timing is already live. [UNK][DD]

Management / maturity / readiness. Finance-team visibility is unknown. [UNK] Audit risk currently reads medium-high, and the next diligence question is billing controls, state compliance, quality metrics, audit history. [DD] The gating issue here is not branding; it is whether billing discipline, staffing durability, authorization quality, and clinical-governance metrics are truly disclosure-ready. [DD]

Strategic synergy. Ranks near the top because it is a real scaled care-delivery platform in an important behavioral-health category, but it stays behind USHV because billing controls, staffing durability, and compliance quality are more likely to drive the final answer than the headline growth story. [FACT-B][INF-S] Behavioral-health platforms can look attractive thematically while still failing

on reimbursement quality, staffing resilience, or compliance depth; the buyer edge only counts if those risks can actually be underwritten. [INF-S] The practical question is whether Soren would be adding underwriting advantage to this asset, not just capital. [INF-S]

Market & transaction feasibility. Current timing evidence is 2019 sponsor entry publicly reported. [FACT-B][INF-S] The public-currency case reads as plausible. [INF-S] The best current route read is that SPAC is plausible, but strategic sale and sponsor-to-sponsor sale may still be more natural default paths. [INF-S] Today that supports active validation, but it still falls short of true sponsor-engagement readiness. [DD]

Principal risks. Billing controls, clinical outcomes, state licensing, staffing, payer mix, quality oversight and public-company controls

Priority diligence focus. Validate revenue scale, payer mix, billing controls, compliance history, clinician staffing, center economics, and owner objectives.

5.5 Hopebridge

Current status: Qualified priority shortlist

Current conclusion: Priority research candidate, but more scale-sensitive than the top two

Field	Current read
Headquarters & subsector	Indianapolis, Indiana — Autism ABA / pediatric behavioral health
Financial view	No confirmed public revenue, EBITDA, or EV. Best public operating-scale proxy: Public materials and sector coverage reference a 100+ center multi-state pediatric autism platform oriented around center-based autism care and early-childhood intervention.
Ownership context	Arsenal Capital Partners-backed; Baird Capital minority interest publicly reported
Current route read	SPAC is plausible if scale confirms; otherwise private recap may be the cleaner route
Evidence profile	FACT coverage 33%

Bottom-line read. The company matters because it combines real care-delivery scale with sponsor ownership in a demand-heavy autism-services category. [FACT-B][INF-S] It ranks below the top provider-services name because reimbursement, staffing, and compliance diligence are more likely to decide the outcome than the headline growth story. [DD]

Basic company characteristics. Hopebridge is based in Indianapolis, Indiana and operates in autism ABA / pediatric behavioral health. [FACT-B] Public materials and sector coverage reference a

100+ center multi-state pediatric autism platform oriented around center-based autism care and early-childhood intervention. [FACT-B] The operating picture is concrete enough to treat the company as a real platform rather than a thematic placeholder. [INF-S]

Financial indicators. Current public sources do not disclose confirmed revenue, EBITDA, or EV. [UNK] The report therefore does not infer valuation or earnings ranges from screening-level signals. [DD] In behavioral health, visible footprint matters because reimbursement quality and staffing durability cannot be underwritten if the operating base itself is still fuzzy. [INF-S] What still cannot be answered from public materials is whether margin quality, leverage, and transaction perimeter would leave a clean SPAC-sized deal. [DD] Next-round diligence should confirm center count revenue per center EBITDA debt and current sponsor expectations. [DD]

Control / ownership structure. Current public materials indicate Arsenal Capital Partners-backed; Baird Capital minority interest publicly reported. [FACT-B] Visible sponsor ownership lowers the odds that this is an orphaned asset and makes a structured process more plausible, but it still does not prove current exit intent. [INF-S] Current public sources do not show a banker-led process, formal sale process, or other direct evidence that transaction timing is already live. [UNK][DD]

Management / maturity / readiness. Finance-team visibility is unknown. [UNK] Audit risk currently reads medium-high, and the next diligence question is clinical outcomes, staff model, payer mix, audit readiness. [DD] The gating issue here is not branding; it is whether billing discipline, staffing durability, authorization quality, and clinical-governance metrics are truly disclosure-ready. [DD]

Strategic synergy. Focused autism-services story with visible center operations; ranked third because the platform looks real but public evidence is thinner than Centria on breadth and transaction context. [FACT-B][INF-S] Behavioral-health platforms can look attractive thematically while still failing on reimbursement quality, staffing resilience, or compliance depth; the buyer edge only counts if those risks can actually be underwritten. [INF-S] The practical question is whether Soren would be adding underwriting advantage to this asset, not just capital. [INF-S]

Market & transaction feasibility. Current timing evidence is 2019 acquisition publicly reported. [FACT-B][INF-S] The public-currency case reads as plausible. [INF-S] The best current route read is that SPAC is plausible if scale confirms; otherwise private recap may be the cleaner route. [INF-S] Today that supports active validation, but it still falls short of true sponsor-engagement readiness. [DD]

Principal risks. Current EBITDA, center productivity, payor economics, clinician supply, outcomes evidence, regulatory exposure and debt

Priority diligence focus. Confirm revenue and EBITDA scale, center maturity, outcomes support, clinician supply, payer mix, leverage, and owner appetite.

5.6 Eating Recovery Center / Pathlight Mood & Anxiety Center

Current status: Provisional carry-forward

Current conclusion: Keep in review, but not yet an actionable shortlist name

Field	Current read
Headquarters & subsector	Denver, Colorado — Eating disorder and mood/anxiety treatment
Financial view	No confirmed public revenue, EBITDA, or EV. Best public operating-scale proxy: Public materials describe a national specialty behavioral-health platform operating under ERC and Pathlight brands with treatment centers plus virtual-care capability.
Ownership context	Apax Partners majority ownership with Oak HC/FT minority/management references publicly reported
Current route read	Strategic or sponsor-led private alternatives currently look at least as likely as SPAC
Evidence profile	FACT coverage 30%

Bottom-line read. This is a strategically relevant specialty platform, but it is still closer to monitored coverage than to an actionable SPAC process. [FACT-B][INF-S] The ranking discount reflects weaker route timing, thinner financial visibility, and heavier diligence burden. [DD]

Basic company characteristics. Eating Recovery Center / Pathlight Mood & Anxiety Center is based in Denver, Colorado and operates in eating disorder and mood/anxiety treatment. [FACT-B] Public materials describe a national specialty behavioral-health platform operating under ERC and Pathlight brands with treatment centers plus virtual-care capability. [FACT-B] The operating picture is concrete enough to treat the company as a real platform rather than a thematic placeholder. [INF-S]

Financial indicators. Current public sources do not disclose confirmed revenue, EBITDA, or EV. [UNK] The report therefore does not infer valuation or earnings ranges from screening-level signals. [DD] In behavioral health, visible footprint matters because reimbursement quality and staffing durability cannot be underwritten if the operating base itself is still fuzzy. [INF-S] What still cannot be answered from public materials is whether margin quality, leverage, and transaction perimeter would leave a clean SPAC-sized deal. [DD] Next-round diligence should confirm payer mix revenue EBITDA outcomes facility economics and comparable deals. [DD]

Control / ownership structure. Current public materials indicate Apax Partners majority ownership with Oak HC/FT minority/management references publicly reported. [FACT-B] Visible sponsor ownership lowers the odds that this is an orphaned asset and makes a structured process more plausible, but it still does not prove current exit intent. [INF-S] Current public sources do not

show a banker-led process, formal sale process, or other direct evidence that transaction timing is already live. [UNK][DD]

Management / maturity / readiness. Finance-team visibility is unknown. [UNK] Audit risk currently reads medium-high, and the next diligence question is payer mix, clinical outcomes, labor model, facility-level economics. [DD] The gating issue here is not branding; it is whether billing discipline, staffing durability, authorization quality, and clinical-governance metrics are truly disclosure-ready. [DD]

Strategic synergy. Differentiated specialty behavioral-health platform with a more specialized clinical story than general ABA peers, but it ranks below the top three because route timing, facility economics, and disclosure readiness are still harder to read from public materials. [FACT-B][INF-S] Behavioral-health platforms can look attractive thematically while still failing on reimbursement quality, staffing resilience, or compliance depth; the buyer edge only counts if those risks can actually be underwritten. [INF-S] The practical question is whether Soren would be adding underwriting advantage to this asset, not just capital. [INF-S]

Market & transaction feasibility. Current timing evidence is 2021 acquisition publicly reported. [FACT-B][INF-S] The public-currency case reads as unclear. [INF-S] The best current route read is that Strategic or sponsor-led private alternatives currently look at least as likely as SPAC. [INF-S] That keeps the company in coverage, but not in the first-call bucket for a SPAC process. [DD]

Principal risks. Outcomes, labor intensity, facility utilization, payor mix, medical necessity, valuation and public-company scrutiny

Priority diligence focus. Confirm scale, reimbursement mix, facility economics, outcomes support, labor model, debt, and management readiness.

5.7 Anne Arundel Dermatology

Current status: Provisional carry-forward

Current conclusion: Useful comparison and backup name, but not yet in the actionable top tier

Field	Current read
Headquarters & subsector	Linthicum, Maryland — Dermatology MSO / specialty physician services
Financial view	No confirmed public revenue, EBITDA, or EV. Best public operating-scale proxy: Public materials describe a multi-state dermatology platform with clinic expansion and repeat bolt-on activity across a fragmented specialty-services market.
Ownership context	Ridgemont Equity Partners-backed

Field	Current read
Current route read	Private recap / sponsor-to-sponsor sale currently appears more natural than SPAC
Evidence profile	FACT coverage 35%

Bottom-line read. This remains a credible coverage name because the platform appears operationally real and strategically relevant. [FACT-B][INF-S] It stays below the active top tier because current public evidence is not yet strong enough on scale, route quality, or public-company readiness. [DD]

Basic company characteristics. Anne Arundel Dermatology is based in Linthicum, Maryland and operates in dermatology MSO / specialty physician services. [FACT-B] Public materials describe a multi-state dermatology platform with clinic expansion and repeat bolt-on activity across a fragmented specialty-services market. [FACT-B] The operating picture is concrete enough to treat the company as a real platform rather than a thematic placeholder. [INF-S]

Financial indicators. Current public sources do not disclose confirmed revenue, EBITDA, or EV. [UNK] The report therefore does not infer valuation or earnings ranges from screening-level signals. [DD] In provider services, visible footprint and clinician density matter because they make it easier to judge whether the platform has real operating depth rather than just roll-up narrative. [INF-S] What still cannot be answered from public materials is whether margin quality, leverage, and transaction perimeter would leave a clean SPAC-sized deal. [DD] Next-round diligence should confirm site count EBITDA M&A pipeline and physician retention economics. [DD]

Control / ownership structure. Current public materials indicate Ridgmont Equity Partners-backed. [FACT-B] Visible sponsor ownership lowers the odds that this is an orphaned asset and makes a structured process more plausible, but it still does not prove current exit intent. [INF-S] Current public sources do not show a banker-led process, formal sale process, or other direct evidence that transaction timing is already live. [UNK][DD]

Management / maturity / readiness. Finance-team visibility is unknown. [UNK] Audit risk currently reads medium, and the next diligence question is CFO/controller, audit history, acquisition pipeline accounting, physician compensation. [DD] The gating issue here is not presentation polish; it is whether site-level economics, physician compensation, bolt-on accounting, and disclosure controls would stand up in a public filing. [DD]

Strategic synergy. Credible specialty-physician-services candidate, but it sits below USHV because the current public narrative feels more like a steady private-equity roll-up than a differentiated public-market story. [FACT-B][INF-S] Soren's edge is easiest to defend in tangible care-delivery models where operating density, specialty positioning, and consolidation logic can be underwritten directly. [INF-S] The practical question is whether Soren would be adding underwriting advantage to this asset, not just capital. [INF-S]

Market & transaction feasibility. Current timing evidence is 2020 acquisition publicly reported. [FACT-B][INF-S] The public-currency case reads as unclear. [INF-S] The best current route read is that Private recap / sponsor-to-sponsor sale currently appears more natural than SPAC. [INF-S] That keeps the company in coverage, but not in the first-call bucket for a SPAC process. [DD]

Principal risks. Possible subscale profile, physician retention, payor/cash-pay mix, acquisition accounting, debt and public-readiness evidence

Priority diligence focus. Confirm EBITDA scale, payer / cash-pay mix, physician economics, acquisition history, leverage, and audit readiness.

6. Reserve / Watchlist Names

The names below remain useful comparables, thematic references, or future candidates, but they do not currently screen ahead of the priority shortlist.

Company	Strategic relevance	Current gating issue	More likely path today	What could move the name up
Cadence	Chronic-care / remote-monitoring model with provider and post-acute relevance.	Ownership timing, revenue scale, and transaction receptivity are not sufficiently visible.	Growth-stage private capital or a later IPO-style route.	Verified revenue, economics, ownership path, and provider-contract durability.
Thyme Care	Oncology navigation remains payer-relevant and strategically attractive.	Public-company scale and transaction timing are not established.	Private growth or strategic route until scale and timing are clearer.	Revenue, lives under management, payer economics, and ownership timing.
Equip Health	Specialty behavioral-health model with outcomes and payer relevance.	Potentially subscale and would require strong clinical-quality disclosure.	Private growth or strategic path unless scale improves materially.	Revenue, payer mix, outcomes data, audit readiness, and investor exit appetite.
Strive Health	Strong value-based kidney-care thesis and close strategic fit with Soren.	Likely valuation, risk-contract complexity, and PIPE burden may exceed a practical first-wave SPAC screen.	IPO-scale or larger-capital private route.	Latest valuation, revenue, lives under management, and PIPE feasibility.

Company	Strategic relevance	Current gating issue	More likely path today	What could move the name up
Carrum Health	Employer specialty-care savings model with a clear cost-containment story.	Standalone public-company size and ownership timing remain unclear.	Private growth, strategic combination, or later-stage public route.	Revenue scale, retention, unit economics, and shareholder timing.
BlueSprig Pediatrics	Autism-services platform with real operating footprint and behavioral-health relevance.	Scale, sponsor context, and transaction timing still need cleaner public confirmation.	Private sponsor route unless valuation and exit timing become clearer.	Center footprint, payer mix, ownership history, and current revenue visibility.
LEARN Behavioral	National autism-services network that fits the current behavioral-health branch of the screen.	Needs better public evidence on ownership, scale, and route timing.	Private recap or sponsor-led route until scale and exit intent are clearer.	Sponsor context, EBITDA scale, reimbursement profile, and compliance visibility.
Action Behavior Centers	Large autism-services candidate with stronger operating density than many digital-health names.	May already sit near or above the practical size boundary for a first-wave Soren SPAC path.	Larger-capital private or strategic route unless structure becomes unusually favorable.	Current valuation, owner timing, revenue confirmation, and financing flexibility.
GI Alliance	Highly relevant specialty physician-services platform from a pure subsector-fit perspective.	Likely too large and structurally heavy for the present SPAC mandate.	Large-cap private, strategic, or IPO-scale route.	Proof of smaller transaction perimeter, financing support, or valuation reset.
Aledade	High-quality value-based primary-care platform.	Likely too large, too competitive, and too complex for Soren's first-wave SPAC screen absent unusual reset conditions.	Large-scale IPO / strategic / late-stage private route.	Valuation, seller willingness, PIPE capacity, and public-readiness evidence.

These companies should remain in coverage. If valuations reset, ownership dynamics change, or public evidence becomes more concrete, one or more could re-enter the active shortlist.

7. Next Diligence Agenda

1. **Harden top-three financial visibility.** Build cleaner revenue, EBITDA, leverage, site-count, provider-count, and acquisition-history support for the active names.
2. **Validate route timing directly.** Move beyond sponsor hold period and test for live sale, recapitalization, banker, refinancing, or strategic-repositioning signals.
3. **Pressure-test SPAC feasibility under redemption scenarios.** For each active name, evaluate funding gap, PIPE need, seller rollover, and pro forma float under low-, base-, and high-redemption cases.
4. **Separate company quality from route quality.** Continue comparing SPAC against sponsor-to-sponsor, strategic sale, private recap, and IPO alternatives so attractive companies are not automatically treated as attractive SPAC candidates.
5. **Close the most important evidence gaps.** Prioritize audit readiness, disclosure maturity, reimbursement / compliance quality, and management readiness — especially where current items remain tagged [UNK] or [DD] .
6. **Produce candidate-specific validation briefs.** For US Heart and Vascular, Centria Healthcare / Centria Autism, Hopebridge, prepare one-page follow-up briefs that distinguish hard public facts, supported inference, remaining unknowns, and diligence-only items.